



Name : **Mast.M THARUN SAI**
Age/Gender : **010Y / Male**
Registration ID : **251330029450**
Ref. By : **Dr M JAGADEESH KUMAR**
Patient Id : **251330029450**

Registration Date Time : 17-Dec-2025 17:57
Study Date Time : 17-Dec-2025 17:57
Report Date Time : 17-Dec-2025 19:52
Modality : MR
Accn No. : 44605637

DEPARTMENT OF RADIOLOGY AND IMAGING SCIENCES 3T CEMRI OF BRAIN

Multiplanar multiple sequence MRI of Brain was performed. 5 ml of Vividscan IV contrast administered. No post injection complications encountered after the scan.

Hx: Posterior fossa tumor for evaluation.

Findings:

Large T2 / FLAIR hyperintense mass lesion showing heterogeneous contrast enhancement is noted in the 4th ventricle predominantly arising from the floor with multiple internal cystic areas / necrosis, measuring 43 x 36 x 39 mm causing significant ventricular obstruction. Abnormal blooming (haemorrhages / calcifications are noted within the lesion). Spectroscopy revealed significantly elevated choline, reduced creatine and NAA levels.

Grossly dilated bilateral lateral and 3rd ventricles with mild periventricular / transependymal edema. Internal flow related artifacts seen. Evans index is ~ 0.43.

Significantly dilated 3rd ventricle and suprasellar recess causing inferior displacement of hypothalamus, mammillary bodies and tuber cinereum. Pituitary stalk is poorly visualised. Significantly reduced mamillopontine distance. No abnormal contrast enhancement seen.

Inferior cerebellar tonsillar herniation below the level of foramen magnum with crowding of the structures, likely secondary to posterior fossa mass and hydrocephalus.

Rest of cerebral parenchyma is normal in signal intensity pattern with normal gray-white matter differentiation.

Rest of brainstem structures and cerebellum are normal.

Cortical sulci and cisterns are normal.

Visualized vascular flow voids are normal.

Dural venous sinuses including cortical veins appear normal in morphology and signal intensity

Sella, pituitary gland and cavernous sinuses are normal.

Calvarium is normal.



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IMPRESSION:

*** Large T2 / FLAIR hyperintense mass lesion showing heterogeneous contrast enhancement is noted in the 4th ventricle predominantly arising from the floor with multiple internal cystic areas / necrosis causing significant ventricular obstruction - possibilities include ependymoma. Differential include medulloblastoma.**

*** Grossly dilated bilateral lateral and 3rd ventricles with mild periventricular / transependymal edema - consistent with obstructive hydrocephalus.**

*** Significantly dilated 3rd ventricle and suprasellar recess causing inferior displacement of hypothalamus, mammillary bodies and tuber cinereum. Pituitary stalk is poorly visualised. Significantly reduced mamillopontine distance. No abnormal contrast enhancement seen - to rule out hypothalamic hamartoma.**

*** Inferior cerebellar tonsillar herniation below the level of foramen magnum with crowding of the structures, likely secondary to posterior fossa mass and hydrocephalus.**

- For clinical / biopsy correlation.

Dr K Dileep Kumar

MBBS, MD

Registration Number: 76496

17th Dec 2025 19:52



Study Sharing Link: <https://tinyurl.com/ykkewxy6>

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Modality : MR
Accn No. : 44605636

DEPARTMENT OF RADIOLOGY AND IMAGING SCIENCES 3T CEMRI OF BRAIN

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